to a single disease. Our health agenda demands attention and impactful action on many fronts beyond COVID-19.

The burden of tuberculosis (TB) is very high, with India accounting for a quarter of the global cases. Multi-drug-resistant (MDR) and extremely drug-resistant (XDR) cases pose a serious challenge. The case finding of TB was affected during the COVID-19 years due to the health system's preoccupation with the tasks of curbing transmission of the SARS-CoV-2 virus, intensifying surveillance for detection of COVID-19 cases, treating them at home or in hospital and undertaking countrywide vaccination in multiple rounds. This meant persons with undiagnosed TB were infecting others. Treatment regimens too were interrupted, as patients could not access healthcare centres.

This setback puts great pressure on India, which had declared its intent to eliminate TB by 2025, ahead of the global target year of 2030. We now need to gear up case detection and treatment, through extensive community engagement and energetic private sector support to the public sector-led national programme. The TB programme should draw lessons from the successful HIV-AIDS control programme, with which it must partner, because TB is a common complication of eroded immunity in AIDS. Similarly, TB control efforts must link with diabetes and tobacco control programmes because of increased incidence in persons with diabetes and smokers.

Among infectious diseases, we have the persistent threat of vector-borne diseases like malaria, dengue and Chikungunya. These will pose greater threats as climate change heats our land and allows mosquitos to spread farther and faster. Apart from COVID-19, new zoonotic diseases will pose threats as breakdown of ecologic barriers will allow viral spillover across species. We need to build robust tracking systems for One Health surveillance across wildlife, captive-bred and free-living veterinary populations and human habitat. Simultaneously, we need to raise the level of preparedness of our health and social systems to deal promptly, effectively and empathetically with any outbreak that carries pandemic potential.

While national aggregate maternal, infant and under-5 child mortality rates are on track to achieve the 2030 targets set by the Sustainable Development Goals (SDGs), we must proactively aim to bridge the six- to sevenfold differences in those rates between high- and low-performing states. We should also strive to improve child nutrition and lower the levels of anaemia in women and children. There are rising rates of overt overweight and obesity as well as the less obvious 'body adiposity' and 'visceral obesity' even among the seemingly non-obese. These are evident across all age groups and pose a high risk of cardio-metabolic diseases. These call for a broader nutrition agenda that addresses all forms of malnutrition.

Presently, the leading causes of death are non-communicable diseases (NCDs). That cluster includes cardiovascular and cerebrovascular diseases (mainly heart attacks and brain strokes), cancers, diabetes, chronic lung, liver and kidney diseases as well as the diseases of eyes, ears, mouth, bones and joints. Many of these cause death or disability in the productive prime of midlife. Mental health disorders are major causes of suffering for affected individuals and their families. NCDs and mental health disturbances are on the rise in India, calling for an intensified health system response. Even during COVID-19 outbreaks, coexisting NCDs markedly enhanced the risk of severe infections, hospitalisation and death. Prevention and control of NCDs and protection of mental health have to be accorded high priority, to protect lives, prevent prolonged disability and safeguard productivity.

PREVENTION AND CONTROL OF NCDs AND PROTECTION OF MENTAL HEALTH HAVE TO BE **ACCORDED HIGH PRIORITY**, TO PROTECT LIVES AND PREVENT DISABILITY

This expansive yet essential agenda can be addressed only if we undertake to strengthen our health system at all levels of care. Primary healthcare is pivotal for good health system performance. Without strengthening both rural and urban primary care, we cannot efficiently control infectious diseases or ensure an energetic platform for pandemic prevention, preparedness and response. Nor can we effectively prevent, detect and treat NCDs or mental health disorders. Maternal and child health are critically dependent on services delivered by primary care.

While ensuring comprehensive and continuous care at the primary level, it is also necessary to connect it bidirectionally to secondary and tertiary care facilities. The Ayushman Bharat programme offers a good platform for building an efficient health system. Its four components—Comprehensive Primary Health Care, Pradhan Mantri Jan Arogya Yojana, Infrastructure Mission and Digital Health Mission—must be seamlessly connected. Investments must be made in developing a multi-layered, multi-skilled health workforce and public health expertise. Centre-state coordination, too, must be harmonious, driven by common commitment to deliver health in all dimensions to the people of India. That resolve must be reflected in substantially raised budgetary allocations in both central and state budgets. Can that begin in 2023? ■